

PREPARED FOR NYLEEN FLORES, MEDELEVATE SOLUTIONS · AUGUST 2026

We check in with your patients before and after their procedure

So they turn up ready, and so what goes wrong at home reaches your nurse rather than an emergency department.

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What we do

Short check-ins on the patient's phone. Every answer is read against our rules, and anything concerning reaches your nurse the same day.

Before the procedure

- Education first
- Prep with reminders
- Fasting, meds, clearance, transport
- Optimization beforehand
- Cancelled slots refilled automatically

The case
on the list

After the procedure

- Recovery check-ins
- Complications caught early
- Recall and post-procedure plans

How we ensure your list runs and patients get the best outcomes

The same case, run twice. Time moves left to right.

	At booking	2 weeks out	48 hours out	Day of	Days 1 to 3	Weeks 1 to 4
Today status quo	Instructions handed over. Nothing more until pre-admission	Staff start phone rounds. Voicemail, callbacks, repeat attempts	Confirmation calls. Anything found now is too late to fix	Cancellation discovered on arrival. Slot lost	Patient calls the office, or drives to an emergency room	Silence read as recovery. Recall slips
With Aescia	Patient onboards to Aescia, which - educates - prepares - verifies needs	Holds, fasting, escort and cardiology clearances confirmed automatically	Readiness verified. A drop-out is known while the slot can still be refilled	Full list, prepared patients	Symptoms triaged. Urgent items to a nurse the same day	Ensures the discharge plan is followed. Recall tracked and booked
Empty slot cost to refill	Trivial	Workable	Hard	Gone		

Pick your problems and price them →

Interactive. Choose what is real at your centers and it totals as you go.
aesciahealth.com/tools/medelevate

What it does for a surgery center

One engine, different content per procedure. A day case is a colonoscopy minus the bowel prep.

Gastroenterology

Fewer wasted slots and better prepared patients

- Bowel preparation is one to three days of unsupervised patient work, and it is where most avoidable failure lives
- Prep quality checked before the patient travels, not discovered on the table
- Cancellations flagged early enough that the slot can still be refilled
- Surveillance recall tracked, so the three-year patient comes back

Everything else on the list

Fewer day-of cancellations and safer recovery at home

- Fasting, escort and anticoagulant holds confirmed rather than assumed
- In one US center's arthroscopy list, 84 percent of short-notice cancellations were patient-side, most often the patient having eaten
- Pain and wound worries routed to a nurse instead of an emergency room
- Specialty modules where the preparation differs. The engine is the same

What I would like from you

The next step is a pilot at one of your centers.

1. **Your read on this as an operations consultant.** Where it breaks, and what an administrator would want to see before letting it near a list.
2. **An introduction to a decision-maker at one of your centers.** Thirty minutes, no commitment beyond hearing it.
3. **What do your centers measure?** Ambulatory Surgical Center Quality Reporting, case volume, block utilization, whatever the board sees each month. If we can move one of those, that is the number worth talking about.

Happy to agree referral terms and a listing on the MedElevate site alongside this.

Live monitoring trial at Royal Prince Alfred Hospital, Sydney · ACTRN12625001425482 · interim readout September 2026

Interactive version: [aesciahealth.com/tools/medelevate](#) · [aesciahealth.com](#) · [james@aesciahealth.com](#)